SACTCHECK · NCCP 00427

# My complete treatment handbook

## Roswell Park modified fluorouracil + folinic acid

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

## Contents


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6. Your treatment passport and symptom record — page 26
7. Wallet card and phone instructions — page 31


SACTCheck v0.84.32 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

# Roswell Park modified fluorouracil + folinic acid

Fluorouracil with folinic acid can reduce recurrence risk after surgery or help control metastatic colorectal cancer. Confirm your treatment aim and planned duration.

**Adjuvant or metastatic colorectal cancer** · NCCP 00427 v6 · Document revised 4 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

## My overall treatment

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

**Treatment**
Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

**Review and next steps**
Your team confirms recovery, response, course length and follow-up; changes need a revised written plan.

## Your treatment schedule

| Weeks 1-6 | Weeks 7-8 | Next cycle |
|---|---|---|
| One injection visit each week | No planned chemotherapy | Restart only after review |

Your team confirms the active phase and dates. Symptoms may occur during or after treatment; there are no guaranteed safe days.

## My tests and checks

| When | What is checked | Timing / reason |
|---|---|---|
| **Before** | **DPD and baseline bloods**<br>Required by regimen | Confirm DPD testing, blood count, kidney and liver results before starting. |
| **During** | **Blood count / kidneys / liver**<br>Required by regimen | Before every weekly treatment. Review symptoms before treatment. |
| **After / review** | **Recovery, scans / markers**<br>Team-agreed plan | Follow the individual cancer plan; this regimen does not prescribe a universal scan or marker interval. |

## Between treatments

- Report fever, sudden illness, diarrhoea, mouth pain or poor intake promptly.
- Symptoms can occur during treatment, between visits and after completion.

Confirm your prescription, supportive medicines, tests and working contact numbers.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**



Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Mouth / eating

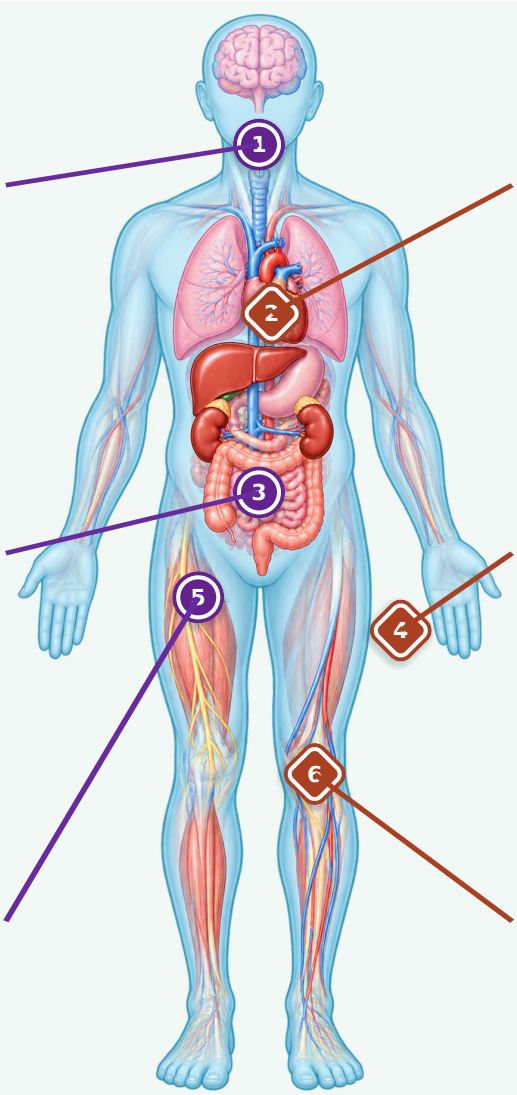
**Fluorouracil**

Report painful ulcers or difficulty eating and drinking.

### 2 Chest / heart

**Fluorouracil**

New chest pain needs urgent assessment, including during an infusion.




### 3 Bowel / hydration

**Fluorouracil**

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

### 4 Hands / feet

**Fluorouracil**

Report painful redness, cracking or peeling that affects daily activity.

### 5 Blood / infection

**Chemotherapy**

Fever, shivers or sudden illness need immediate oncology advice.

### 6 Energy / daily life

**Treatment**

Report fatigue that limits usual activity; ask for practical and nutrition support.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

# Keep your treatment details to hand

**Adjuvant or metastatic colorectal cancer** · NCCP 00427 v6 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

**My Treatment Card** SACTCheck

Name: _____

**Roswell Park modified fluorouracil + folinic acid**

Weekly x6, then 2 weeks off; confirm prescribed variant.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

**BACK / TREATMENT RECORD**

**My cycle record**

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

NCCP 00427 · VERSION 6

# Your treatment, explained

**Adjuvant or metastatic colorectal cancer** · NCCP 00427 v6 · Document revised 4 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Roswell Park modified fluorouracil + folinic acid

56-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

## When to get help

### Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

### Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

**Open your CUH contact / treatment card**

Sources: nccp

YOUR COMPLETE REFERENCE

# Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.


- How each drug works
- Your treatment plan
- Your 56-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Your treatment aim and duration
- Tests and safety
- Recognise symptoms and act
- Daily life and support
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources


## UNDERSTAND YOUR TREATMENT

# Your medicines and what they do

Fluorouracil with folinic acid can reduce recurrence risk after surgery or help control metastatic colorectal cancer. Confirm your treatment aim and planned duration.

**CHEMOTHERAPY MODULATOR**

## Folinic acid

Calcium leucovorin



### Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

### What to watch for

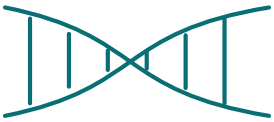
The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

Sources: nccp

CHEMOTHERAPY

### Fluorouracil

5-FU



**Disrupts cell building blocks**

Fluorouracil interferes with DNA and RNA, the instructions cells need to grow. The same drug has different schedules; follow the prescription for this regimen.

Drug enters cell → DNA / RNA disrupted → Growth slows

**What to watch for**

Report diarrhoea, mouth sores, painful hands or feet and unexpected severe early symptoms. Chest pain needs urgent assessment even after a normal DPD test.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

**Explore your existing anatomy and side-effect map**

THE BIGGER TREATMENT PLAN

# Your treatment plan

ADJUVANT OR METASTATIC COLORECTAL CANCER

### Prescribed treatment course




Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

## Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

| Check | Before starting | Treatment visits | Between visits | Review | After treatment |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** | Baseline tests and DPD status | Before every weekly treatment | Extra tests if unwell or dehydrated | Results guide any delay or change | Follow unresolved abnormalities |
| **Symptoms and medicines** | Review medicines, cardiac history and usual function | Review bowel, mouth, skin and infection symptoms | Use urgent contact plan | Confirm recovery and next dose | Agree recovery care |
| **Scans / tumour markers** | Confirm disease assessment | According to individual plan | New symptoms may bring review forward | Consultant-directed response assessment | Individual follow-up |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00427 v6 official regimen

THE WHOLE TREATMENT JOURNEY

# Roswell Park modified fluorouracil + folinic acid: treatment and checks

### Adjuvant or metastatic colorectal cancer

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Treatment visits | Between visits | Review | After treatment |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** NCCP 00427 | Baseline tests and DPD status | Before every weekly treatment | Extra tests if unwell or dehydrated | Results guide any delay or change | Follow unresolved abnormalities |

| Care / check | Before starting | Treatment visits | Between visits | Review | After treatment |
| --- | --- | --- | --- | --- | --- |
| **Symptoms and medicines** NCCP 00427 | Review medicines, cardiac history and usual function | Review bowel, mouth, skin and infection symptoms | Use urgent contact plan | Confirm recovery and next dose | Agree recovery care |
| **Scans / tumour markers** NCCP 00427 | Confirm disease assessment | According to individual plan | New symptoms may bring review forward | Consultant-directed response assessment | Individual follow-up |

## What each check is for

1

### Confirm your personal plan

Before starting

Fluorouracil with folinic acid can reduce recurrence risk after surgery or help control metastatic colorectal cancer. Confirm your treatment aim and planned duration.

**Ask / act:** Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00427

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

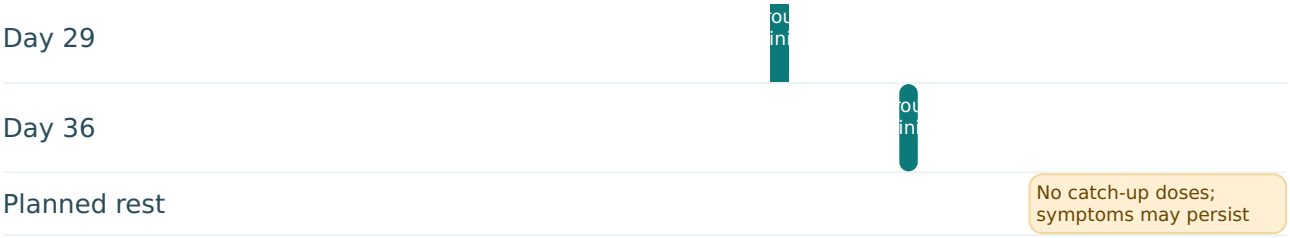
Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32

## Day 1 → Day 56: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.






**The written plan comes first:** Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

**Symptoms can continue:** Contact oncology for fever, sudden illness, diarrhoea, mouth soreness or inability to drink, during treatment or the break.

**Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00427 v6 official regimen ↗

**There are no guaranteed “safe days”.** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

**Read the whole cycle as text**

### Day 1–42: Six weekly treatment visits

Attend on Days 1, 8, 15, 22, 29 and 36; both medicines at each visit. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

### Day 43–56: Planned two-week break

No catch-up doses; symptoms can continue during the break. Confirm the next plan. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

PRACTICAL TREATMENT GUIDE

# Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

### Before treatment

Confirm the treatment aim, duration, DPD result, blood tests, prescription and contact numbers.



STEP 2

### Visits and access

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.This regimen does not require a home infusion pump.



STEP 3

### If treatment is delayed

Ask for revised written treatment and support-medicine instructions. Do not make up missed doses or carry an old plan into a new phase.



STEP 4

### Getting help

Keep the wallet card and medicine list available. Contact the team promptly for infection, dehydration, chest symptoms or significant toxicity.

**Open practical treatment guide** · **Printable practical guide**

Sources: NCCP 00427 v6 official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

# Support around the medicines

### 1 Supportive medicines

Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.

### 2 Mouth, bowel and skin care

Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.

### 3 Medicine checks

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

Sources: nccp

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TREATMENT REFERENCE · 01

# Your treatment aim and duration

### Why this treatment?

Fluorouracil with folinic acid can reduce recurrence risk after surgery or help control metastatic colorectal cancer. Confirm your treatment aim and planned duration.

Sources: nccp

### Your course

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

Sources: nccp

### Choices

Discuss the expected benefit, burdens, alternatives and how treatment fits your priorities. You can ask for more time or another explanation before deciding.

Sources: nccp

TREATMENT REFERENCE · 02

# Tests and safety

### DPD before fluorouracil

Low DPD activity can cause severe toxicity. Testing helps guide the plan but cannot prevent every reaction. Unexpected severe early symptoms need urgent advice.

Sources: nccp

### Blood tests

Before every weekly treatment: blood count, renal and liver profile; baseline DPD. Scans/markers follow the consultant plan.

Sources: nccp

### Medicines and interactions

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

Sources: nccp

TREATMENT REFERENCE · 03

## Recognise symptoms and act

### Mouth / eating

Report painful ulcers or difficulty eating and drinking.

Sources: nccp

### Chest / heart

New chest pain needs urgent assessment, including during an infusion.

Sources: nccp

### Bowel / hydration

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

Sources: nccp

### Hands / feet

Report painful redness, cracking or peeling that affects daily activity.

Sources: nccp

### Blood / infection

Fever, shivers or sudden illness need immediate oncology advice.

Sources: nccp

### Energy / daily life

Report fatigue that limits usual activity; ask for practical and nutrition support.

Sources: nccp

TREATMENT REFERENCE · 04

# Daily life and support

### Food and fluids

Aim for tolerable meals and fluids. Report weight loss or poor intake; ask for dietetic support.

Sources: nccp

### Activity and rest

Adjust activity and rest to your energy. Report fatigue that limits normal tasks.

Sources: nccp

### Fertility and sexual health

Ask about fertility preservation before treatment and the contraception and breastfeeding plan for all medicines. Report a possible pregnancy promptly.

Sources: nccp

### Support for you and your family

Ask about counselling, psycho-oncology, practical support, transport and financial advice. Bring someone to discussions if helpful.

Sources: nccp

## Understanding the evidence

Ask which evidence matches your treatment aim.

NCCP cites NSABP C-03, which studied postoperative fluorouracil/leucovorin in colon cancer. This historical evidence does not establish an individual benefit for every metastatic setting or every modified folinic-acid schedule.

Sources: nccp · trial

**PREPARE FOR THE CONVERSATION**

# Questions worth asking

**Which treatment plan applies to me?**

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan. Your team confirms any modification.

**Do more side effects mean treatment is working?**

No. Side effects do not reliably measure response. Report symptoms and follow the agreed review plan.

**Who do I contact overnight?**

Ask the team to replace example numbers with working daytime, out-of-hours and backup contact routes before leaving.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

SOURCE TRANSPARENCY

# Where this information comes from

Regimen schedule: NCCP 00427 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00427 v6 official regimen
- NCCP NCIS antiemetic guidance V8
- NSABP C-03 primary trial report

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-04.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

## Written source addresses

https://healthservice.hse.ie/documents/6466/427_Roswell_Park_Modified.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/8410113/

BASELINE → TREATMENT → FOLLOW-UP

# Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

| Check | Before starting | Treatment visits | Between visits | Review | After treatment |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** | Baseline tests and DPD status | Before every weekly treatment | Extra tests if unwell or dehydrated | Results guide any delay or change | Follow unresolved abnormalities |
| **Symptoms and medicines** | Review medicines, cardiac history and usual function | Review bowel, mouth, skin and infection symptoms | Use urgent contact plan | Confirm recovery and next dose | Agree recovery care |
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The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00427 v6 official regimen

THE WHOLE TREATMENT JOURNEY

# Roswell Park modified fluorouracil + folinic acid: treatment and checks

### Adjuvant or metastatic colorectal cancer

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

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**Treatment, tests and reviews across the course**

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# What each check is for

1

### Confirm your personal plan

Before starting

Fluorouracil with folinic acid can reduce recurrence risk after surgery or help control metastatic colorectal cancer. Confirm your treatment aim and planned duration.

**Ask / act:** Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00427

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32

# Roswell Park modified fluorouracil + folinic acid

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Fluorouracil treatment day** — Low
  Folinic acid + Fluorouracil

https://sactcheck.com/patient/00427/supportive-care.html

# Roswell Park modified fluorouracil + folinic acid

NCCP 00427 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

### Before treatment

Confirm the treatment aim, duration, DPD result, blood tests, prescription and contact numbers.

STEP 2

### Visits and access

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.This regimen does not require a home infusion pump.

STEP 3

### If treatment is delayed

Ask for revised written treatment and support-medicine instructions. Do not make up missed doses or carry an old plan into a new phase.

STEP 4

### Getting help

Keep the wallet card and medicine list available. Contact the team promptly for infection, dehydration, chest symptoms or significant toxicity.

Sources: NCCP 00427 v6 official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

# Roswell Park modified fluorouracil + folinic acid

## A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Fluorouracil and folinic acid are given once weekly for six weeks, then two weeks off: Days 1, 8, 15, 22, 29 and 36 of a 56-day cycle. Adjuvant treatment is three cycles (24 weeks). A four-dose/six-week variant requires a separate consultant plan.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

**Emergency: 112 / 999.** Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

**Contact oncology immediately** for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital’s instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your out-of-hours pathway / nearest ED**

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00427/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

# Roswell Park modified fluorouracil + folinic acid

Cycle number: _____ Start date: _____ Next review: _____

### Folinic acid · Chemotherapy modulator

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | ☐ None | ☐ Mild soreness; eating normally | ☐ Painful; soft food needed: call | ☐ Cannot eat / drink enough: urgent review | ☐ Major breathing difficulty: 112/999 |

### Fluorouracil · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | ☐ None | ☐ Mild soreness; eating normally | ☐ Painful; soft food needed: call | ☐ Cannot eat / drink enough: urgent review | ☐ Major breathing difficulty: 112/999 |
| **Fatigue** | ☐ Usual energy | ☐ Tiredness improves with rest | ☐ Rest does not help; usual tasks affected: call | ☐ Self-care difficult: urgent review | ☐ Collapse: 112/999 |
| **Hand-foot soreness** | ☐ None | ☐ Redness or tingling without pain | ☐ Pain limits usual tasks: call | ☐ Pain limits washing / dressing: urgent review | ☐ Acute severe illness: emergency help |

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Roswell Park modified fluorouracil + folinic acid

Cycle number: _____ Start date: _____ Next review: _____

### Folinic acid · Chemotherapy modulator

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | ☐ None | ☐ Mild soreness; eating normally | ☐ Painful; soft food needed: call | ☐ Cannot eat / drink enough: urgent review | ☐ Major breathing difficulty: 112/999 |

### Fluorouracil · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
| **Mouth soreness / ulcers** | ☐ None | ☐ Mild soreness; eating normally | ☐ Painful; soft food needed: call | ☐ Cannot eat / drink enough: urgent review | ☐ Major breathing difficulty: 112/999 |
| **Fatigue** | ☐ Usual energy | ☐ Tiredness improves with rest | ☐ Rest does not help; usual tasks affected: call | ☐ Self-care difficult: urgent review | ☐ Collapse: 112/999 |
| **Hand-foot soreness** | ☐ None | ☐ Redness or tingling without pain | ☐ Pain limits usual tasks: call | ☐ Pain limits washing / dressing: urgent review | ☐ Acute severe illness: emergency help |

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Roswell Park modified fluorouracil + folinic acid

Cycle number: _____ Start date: _____ Next review: _____

### Folinic acid · Chemotherapy modulator

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
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### Fluorouracil · Chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
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# Roswell Park modified fluorouracil + folinic acid

Cycle number: _____ Start date: _____ Next review: _____

### Folinic acid · Chemotherapy modulator

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
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| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
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### Fluorouracil · Chemotherapy

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| **Diarrhoea** | ☐ Usual pattern | ☐ Up to 3 extra stools above normal | ☐ 4–6 extra stools/day or at night: call | ☐ 7+ extra stools/day or dehydration: urgent review | ☐ Collapse / severe dehydration: 112/999 |
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Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Keep your treatment details to hand

**Adjuvant or metastatic colorectal cancer** · NCCP 00427 v6 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Roswell Park modified fluorouracil + folinic acid**

Weekly x6, then 2 weeks off; confirm prescribed variant.

Phase: _____ Cycle plan: team to confirm

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

**BACK / TREATMENT RECORD**

## My cycle record

Rolling record — follow your agreed treatment plan.

| Cycle | Date / initials |
|---|---|
| | |
| | |
| | |
| | |

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00427 v6 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.